A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

18 May 2026

Dr Andrew Ouchtomsky
Clifton Hill Medical Group

Dear Dr Ouchtomsky,

Re: Ms Catherine McKibbin
DOB: 28/02/1963 Phone: 0449 678 622
Address: Unit 3, 182 St Georges Road, NORTHCOTE, VIC, 3070
Email: katemckibbin@bigpond.com

Summary

- **Lymphocytic colitis with likely concurrent irritable bowel syndrome.**
- **Acute flare of symptoms including faecal incontinence following a stressful life event.**
- **Mildly elevated MCV for further investigation with B12, folate, iron studies, and thyroid function.**
- **For escalation of Entocort and potential initiation of low-dose amitriptyline.**
- **Action required: Please facilitate blood tests (B12, folate, iron studies, and TFTs).**

I had the pleasure of conducting a review with Catherine McKibbin. It has been nearly two years since our last consultation. Catherine recently experienced a significant flare of her bowel symptoms approximately five days ago, triggered by an acutely stressful event at work. Her symptoms have been characterized by increased stool frequency, loose consistency, and episodes of faecal incontinence. Prior to this flare, Catherine was maintained on Entocort (budesonide) at a dose of one tablet every second or third day. In response to the current flare, she has increased this to two tablets daily. Interestingly, she noted that even when her lymphocytic colitis was ostensibly stable, she rarely achieved a truly firm bowel movement. This suggests to me that she likely has two concurrent pathologies: lymphocytic colitis and a superimposed functional component in the form of irritable bowel syndrome. We discussed how stress impacts the autonomic nervous system of the gut, increasing colonic contractility and rectal sensitivity.

Catherine raised concerns regarding the role of steroids in cataract formation. I have reassured her that while systemic steroids are a known contributor, budesonide is unique due to its high first-pass metabolism, with 99% being cleared by the liver. Consequently, systemic absorption is minimal (approximately 1%), which significantly mitigates the risk of systemic side effects like cataracts compared to traditional corticosteroids.

Recent pathology from Melbourne Pathology shows a normal CRP of 1, normal kidney function, and normal liver function. However, her MCV is mildly elevated. While Catherine's alcohol intake may be a contributing factor, I would like to exclude other causes. To that end, I have requested that she undergo testing for B12, folate, iron studies, and thyroid function. I have also noted that a faecal calprotectin is often of limited utility in microscopic colitis compared to other forms of inflammatory bowel disease, and I will rely on clinical response and potentially future endoscopic assessment if symptoms persist.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Impression and Plan My plan is for Catherine to continue the increased dose of Entocort for the next two weeks. If she does not see a substantial improvement in her bowel habits, I have suggested we address the autonomic gut dysfunction by initiating low-dose amitriptyline (Endep). We discussed that at doses of 10 mg to 35 mg, it acts as a neuromodulator to slow transit, dry secretions, and reduce visceral hypersensitivity. I have confirmed she has no history of glaucoma. I have provided a prescription for amitriptyline to be used at her discretion should the Entocort escalation fail to provide adequate relief. I have also advised she may use Gastro-Stop as required.

I have asked Catherine to book a review with me in two months' time if she commences the amitriptyline, so we can assess its efficacy and monitor for side effects like sedation or dry mouth. But in the interim her preference was to see me as needed. Thank you again for re-referring Catherine

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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